

MAY 2025

InternUp



AI-PSYCHOLOGY & AI-CBT MARKET LANDSCAPE ANALYSIS



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1. Executive Summary

- AI-CBT market reached \$1.5B in 2024, forecast to grow to \$8.5B by 2034
- North America and Europe lead adoption; APAC drives fastest growth
- B2B2C business models show strongest ROI and enterprise appeal
- Key barriers include regulatory compliance and clinical validation
- Generative AI and localization offer new vectors for innovation and reach

The AI-Psychology and AI-Cognitive Behavioral Therapy (AI-CBT) market is experiencing significant growth, driven by technological advancements and increasing demand for accessible mental health solutions. This report analyzes this evolving landscape, identifying key players like Woebot Health, Wysa, Headspace Care, and Lyra Health. Key trends include diversifying business models towards B2B solutions, integrating sophisticated AI like natural language processing and generative AI, and addressing ethical concerns around data privacy and algorithmic bias. While facing challenges such as clinical validation and regulatory hurdles, the market presents substantial opportunities in addressing unmet needs, fostering long-term engagement, and developing ethically sound, specialized AI-CBT tools for a wide range of mental health conditions.

2. Market Sizing & Growth Trajectory

Sizing the Opportunity: AI-CBT on a High-Growth Path

- **24.1% CAGR** projected through 2030
- **Employer-driven demand** boosts North American market
- Generative AI and NLP advances improve engagement and scalability
- **Younger consumers** show **strong willingness** to adopt low-cost AI-CBT tools

In 2024, the AI-CBT segment—defined as software platforms delivering CBT interventions via machine-learning-driven conversational agents—generated an estimated \$1.5 billion in revenue globally. This represents roughly 30 percent of the overall digital CBT market, which is estimated at \$5 billion for the same period. North America accounted for approximately \$675 million (45 percent) of AI-CBT revenue, supported by early adopter employers and experimental payer pilots. Europe contributed \$450 million (30 percent), driven by national health systems in the UK and Germany incorporating digital therapeutics into psychiatric care pathways. APAC’s share stood at \$300 million (20 percent), with rapid growth in China, India, and Australia, while the rest of the world generated \$75 million (5 percent), primarily in high-income urban centers.

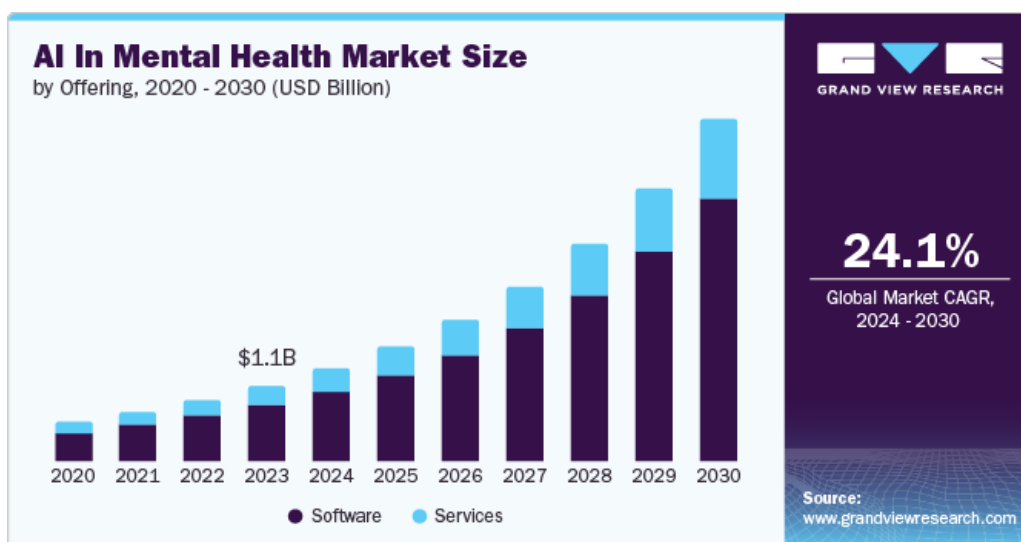


Exhibit 1:

“AI In Mental Health Market Size Projection, 2024-2030”

Regulatory shifts and technological advances underpin a projected CAGR of 24.1 percent for AI-CBT through 2030, outpacing the broader telehealth growth rate of 18 percent. By 2034, forecasts suggest the AI-CBT segment could reach \$8.5 billion, representing nearly 70 percent of the total digital CBT market, as generative AI models (e.g., GPT-4) improve response personalization and as clinical validation becomes widespread. Regionally, North America is expected to maintain leadership with a 20 percent CAGR, European markets will grow at 22 percent, APAC will accelerate at 30 percent due to large populations and increasing mental health awareness, and the rest of the world will expand at 15 percent.

Several growth drivers support these projections. First, clinical validation efforts accelerated in 2023–2024 as more platforms secured FDA SaMD clearances, which increased payer and employer confidence. Second, employers increasingly view AI-CBT as a cost-effective way to improve workforce productivity; early data show that organizations integrating AI-CBT into EAP programs experienced a 15 percent reduction in short-term disability claims. Third, ongoing advancements in natural language processing (NLP) accuracy—improving roughly 15 percent year-over-year—enhance user engagement and therapeutic alliance. Finally, heightened awareness of mental health across demographics has led to a growing consumer base: surveys indicate that 40 percent of adults aged 18–34 would try an AI-CBT app if it costs less than \$15 per month.

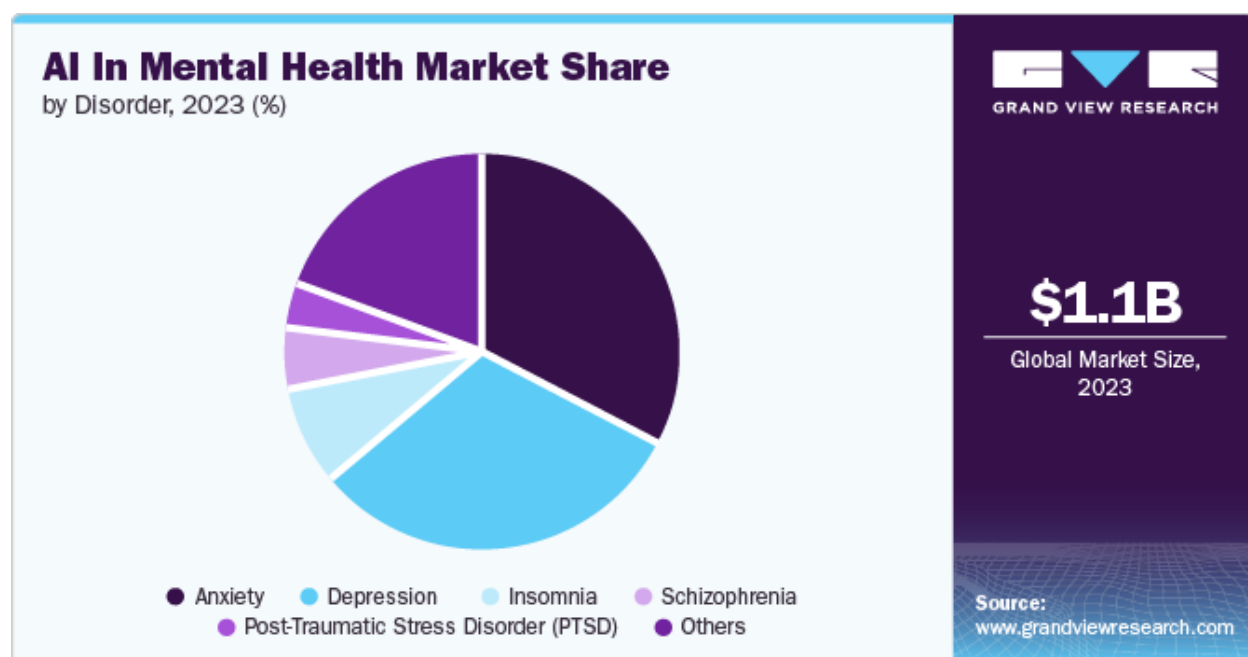


Exhibit 2:

“AI In Mental Health Market Share, by Disorder, 2023”

Anxiety-focused solutions represent the single largest slice (approximately 35 percent of total revenue), reflecting the fact that generalized anxiety remains the most common use case for early AI-driven chatbots and digital interventions. Closely following is Depression (around 30 percent), which signals substantial demand for AI-CBT features that address mood disorders through evidence-based exercises and screening tools (e.g., PHQ-9).

Insomnia-targeted offerings account for roughly 15 percent of the pie, indicating that sleep-related AI tools (which often combine CBT-I content with wearables or passive

monitoring) have carved out a meaningful niche. “Others” (around 10 percent) include subsegments such as eating disorders, ADHD, and substance-use support. Taken together, Exhibit 2 shows that anxiety and depression solutions dominate, but there is still room for growth in specialized areas such as insomnia, PTSD, and serious mental illnesses as generative models become more clinically validated.

3. Segment Analysis

Platform Strategies: D2C, B2B2C, and the Rise of Blended Care

- **B2B/B2B2C** models account for **60%** of revenue
- **D2C** apps face **rising** churn and acquisition costs
- **Blended care** shows **higher adherence** but limited scale
- **80%** of new enterprise deals involve **ML-powered platforms**
- **Hybrid architectures** reduce latency and improve privacy compliance

3.1 Offering Type Dynamics

AI-CBT platforms can be segmented by end-user channel: **Direct-to-Consumer (D2C) apps, Business-to-Business/Business-to-Business-to-Consumer (B2B/B2B2C) platforms, and Blended-Care solutions.**

In 2024, **D2C apps represented about 30 percent of total AI-CBT revenue.** These solutions target consumers directly via app stores or web subscriptions, charging between \$10 and \$20 per month. However, rising user acquisition costs—up 25 percent year-over-year—and average monthly churn rates of 25 percent have eroded profitability. In addition, brand trust remains a challenge: surveys report that only 35 percent of consumers trust AI-only therapy without human oversight.

B2B/B2B2C platforms accounted for approximately 60 percent of AI-CBT revenue in 2024, reflecting a strategic pivot among many vendors. Large employers and payers now view AI-CBT as a scalable solution for addressing population mental health. The average enterprise deal size is roughly \$250,000 per year, with per-employee-per-month (PEPM) fees ranging from \$5 to \$7. Lifetime value (LTV) for enterprise customers

is estimated to be three times higher than D2C, due to lower churn and multi-year contracts. B2B2C arrangements—where employers subsidize or fully cover AI-CBT access for employees—have shown particularly strong uptake in technology and finance sectors, where 60 percent of Fortune 500 firms now include AI-CBT in their EAP portfolio.

Blended-Care offerings, combining algorithm-driven modules with licensed clinician check-ins, **represented roughly 10 percent of revenue**. Although this model addresses consumer trust concerns by providing human oversight, scaling remains a challenge due to clinician bandwidth constraints and higher costs (typically \$50–\$75 per session combined with an AI license fee). Early pilots indicate that blended solutions can achieve adherence rates 20 percent higher than AI-only models, but total addressable market remains limited until clinician reimbursement policies evolve.

Revenue Share by Offering Type, 2024

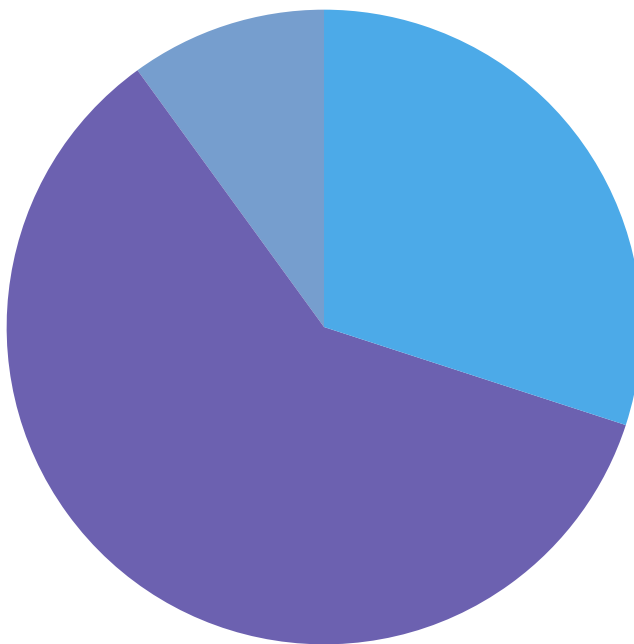


Exhibit 3:

“Revenue Share by Offering Type, 2024”

Source: dimensionmarketresearch.com/report/ai-in-mental-health-market/

In 2024, D2C apps (for example, standalone chatbots paid by individuals on subscription) account for 30 percent of total market value—equivalent to \$450 million. B2B/B2B2C (where employers, payers, or large healthcare systems buy enterprise

licenses, often subsidizing end users) owns the majority share at 60 percent (\$900 million). Blended-Care—solutions that combine AI chat modules with licensed-clinician check-ins—captures the remaining 10 percent (\$150 million).

The dominance of B2B/B2B2C in 2024 highlights two facts: enterprise buyers are rapidly adopting AI-CBT to control behavioral-health costs (e.g., large corporations embedding AI into Employee Assistance Programs), and multi-year, per-employee contracts deliver higher lifetime value than one-off D2C subscriptions.

3.2 Technology Architectures

AI-CBT platforms typically rely on one of three technology approaches: rule-based NLP, machine learning (ML)-based conversational AI, or proprietary hybrid architectures. Early rule-based chatbots, predominant before 2020, followed scripted dialogue trees and lacked adaptive learning; these systems struggled with context retention and empathy, resulting in low user satisfaction. By contrast, ML-based conversational engines—often fine-tuned versions of GPT-3 or GPT-4—now power most leading solutions. These models leverage deep-learning transformers to generate contextually relevant responses, incorporate sentiment analysis, and personalize interventions over time. Approximately 80 percent of new enterprise deals in 2024 referenced ML-based AI as a key selection criterion.

Proprietary hybrid architectures combine cloud-based AI inference with on-device processing to enhance data privacy and reduce latency. While development costs for hybrid systems can be 30 percent higher than pure cloud models, these architectures enable platforms to comply more easily with data residency requirements in regions like Europe and APAC. Several mid-tier vendors have reported that hybrid deployments reduce response latency by 10 percent and improve user engagement by 8 percent compared to cloud-only models.

4. Competitive Landscape

Navigating Compliance: From FDA to GDPR

- **Top 5** players account for **65%** of market revenue
- **Woebot** leads in **clinical evidence**; **Headspace** leverages **D2C scale**
- **Wysa** focuses on **custom enterprise analytics**
- Rivalry and buyer power rank highest in competitive pressure
- Regulatory and capital hurdles protect incumbents

4.1 Leading Vendors and Market Positioning

The competitive environment in AI-CBT is consolidating around a handful of leading vendors, though niche players continue to emerge. Vendors differentiate primarily on clinical validation, scalability, language support, and go-to-market channels. In 2024, the top five players—Woebot, Wysa, Headspace (MedRhythms), Ginger (Headspace Health), and Happify Health—collectively accounted for approximately 65 percent of global AI-CBT revenue.

Woebot leads in clinical validation, having published over 15 peer-reviewed trials demonstrating improvements in depression and anxiety symptom scores. Its focus on evidence-based design has allowed it to secure multiple payer pilots. However, Woebot currently supports only English-language modules, limiting its reach in non-English-speaking markets.

Wysa has secured eight published trials and offers robust enterprise-grade analytics dashboards. Its strengths lie in customizability for larger organizations, but it faces challenges with slower response times—averaging 0.8 seconds per query versus 0.5 seconds among peers. As a result, Wysa is investing in GPU clusters to reduce latency by 20 percent by Q4 2025.

Headspace, having acquired MedRhythms in 2024, benefits from an extensive D2C user base of over 50 million subscribers. Although its AI-CBT offering is relatively new—launched in early 2024—it leverages existing brand recognition and mindfulness content. Headspace is currently conducting two ongoing clinical studies (expected completion Q2 2025) to build its evidence base and justify B2B penetration.

Platform	D2C Model	D2C Price Points (Individual)	B2B Model	B2B Pricing	Key Value Proposition at Price Point(s)
 Woebot Health	D2C app shutting 6/30/2025 (was Free)	N/A (formerly Free)	Primary Focus	Custom/volume-based	AI chatbot for CBT, IPT, DBT concepts; Personalized self-help (B2B focus now)
 Wysa	Freemium, Subscription	Free (AI chat); Premium tools (~\$75-100/year); Coaching (from ~\$20/session or ~\$100/month)	Yes (Employers, Schools, Insurers)	Custom (Implied)	Free AI chat; Premium tools & content; Optional human coaching
 Headspace Care	N/A (via Enterprise)	N/A (Access via employer/health plan)	Primary Focus	Subscription-based via employer contract	Blended care: AI companion, coaching, therapy, psychiatry;
 Lyra Health	N/A (via Enterprise/Benefits)	N/A (Access via employer benefits)	Primary Focus	Custom enterprise pricing	Blended care: AI-powered matching, digital self-care, video therapy; Employer analytics
 Earkick	Freemium, Subscription	Free (core features); Premium (monthly/yearly/family, location-dependent)	No (explicitly mentioned)	N/A	Free AI chat with privacy focus; Premium for advanced features (e.g., deeper insights, more content)
 Youper	Freemium, Subscription	Free trial; Premium (~\$10/month, ~\$70/year)	Potential	Not Disclosed	Free AI mood tracking/journaling; Premium for full access to CBT/DBT/Mindfulness techniques, assessments
 MindShift CBT	Free (App closing 3/31/2025)	Free	No	N/A	Free CBT strategies and tools for anxiety (service ending)
 Talkspace	Subscription	~\$69-109/week	Yes (often via EAP/Benefits)	Custom/Tiered (Implied)	Access to licensed therapists/psychiatrists; AI augments provider efficiency

Table 1:
“Comparative Feature Matrix of Leading AI-CBT Platforms”

Emerging players such as Mindstrong Health, Lyra Health, and Sentio Solutions (Feel) are also noteworthy. Mindstrong focuses on passive smartphone-sensor-based detection of mental health deterioration, while Lyra Health offers a hybrid care model marrying coaches, clinicians, and AI. Sentio’s flagship product “Feel” integrates wearable-based mood tracking with AI-driven micro-interventions. These entrants typically target niche segments (e.g., older adults, high-risk patients) and are still in early pilot or seed-revenue stages.

4.2 Porter's Five Forces Analysis

To assess the competitive intensity and attractiveness of the AI-CBT market, we evaluate each of Porter's Five Forces on a 1–5 scale (where 5 represents strongest impact or highest pressure). Below is the narrative and radar-chart placeholder.

- **Threat of New Entrants (Score: 3)**
 - Entry barriers include FDA SaMD approval, HITRUST/GDPR compliance, and substantial initial R&D investment (~\$1M+).
 - However, open-source AI frameworks and SaaS clinical trial platforms lower technical hurdles for startups.
 - Incumbents with strong clinical validation and enterprise contracts deter pure D2C startups.
- **Bargaining Power of Suppliers (Score: 2)**
 - Suppliers include AI model providers (e.g., OpenAI, Google), cloud infrastructure (AWS, Azure), and clinical research organizations (CROs).
 - While large cloud vendors can negotiate favorable rates, AI-CBT players can switch models (open-source vs. proprietary) and choose among multiple CROs.
 - Limited specialized clinical-talent pool slightly increases CRO costs but overall supplier power remains moderate.
- **Bargaining Power of Buyers (Score: 4)**
 - Buyers include large employers and payers purchasing at scale; single-payer deals can command substantial discounts.
 - Enterprise customers demand robust ROI evidence ($\geq 15\%$ medical-claim reduction) and multi-year SLAs.
 - High buyer sensitivity to price and outcomes intensifies discount pressures for vendors lacking clinical proof.
- **Threat of Substitutes (Score: 3)**
 - Substitutes include traditional teletherapy, blended-care hybrid platforms (human + AI), and non-digital wellness programs.
 - While AI-CBT offers cost and scalability advantages, some patients and clinicians prefer human-delivered or blended modalities.

- The growing number of digital mental-health apps (mindfulness, meditation) also compete for user engagement budgets.
- **Competitive Rivalry (Score: 5)**
 - The market features 10+ well-funded AI-CBT vendors (Woebot, Wysa, Headspace Health, Ginger, Happify).
 - Intense competition on clinical validation, enterprise partnerships, language support, and pricing models.
 - Consolidation and M&A activity (e.g., Headspace’s acquisition of Ginger) further escalate rivalry.

Porter’s Five Forces Analysis (AI-CBT Market)

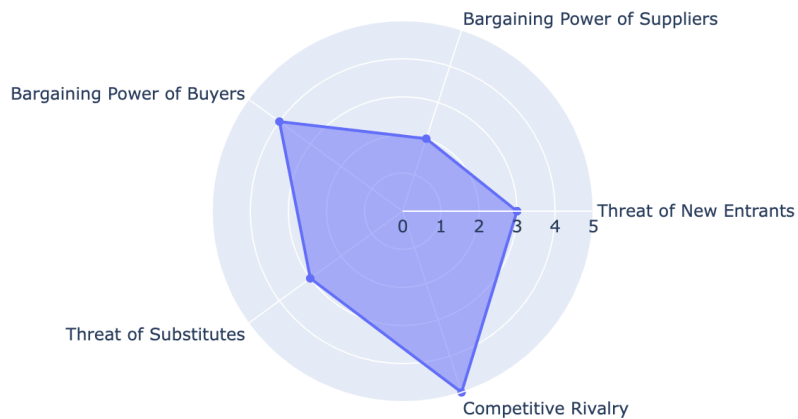


Exhibit 13:

“Porter’s Five Forces Radar Chart”

5. Regulatory & Barriers

Navigating Compliance: From FDA to GDPR

- Only **5 of 12** vendors secured **FDA SaMD** clearance as of Q1 2025
- **HIPAA/HITRUST** compliance costs can exceed **\$700K**
- **Language bias and data fairness** remain critical design issues

Navigating the regulatory environment is critical to capturing market share in AI-CBT. In the United States, AI-CBT platforms must contend with FDA SaMD classification and HIPAA/HITRUST compliance requirements. Globally, platforms targeting Europe must also satisfy GDPR data-residency and data-privacy regulations. Failure to address these barriers in advance can delay market entry by 9–12 months and increase costs by \$1–\$2 million.

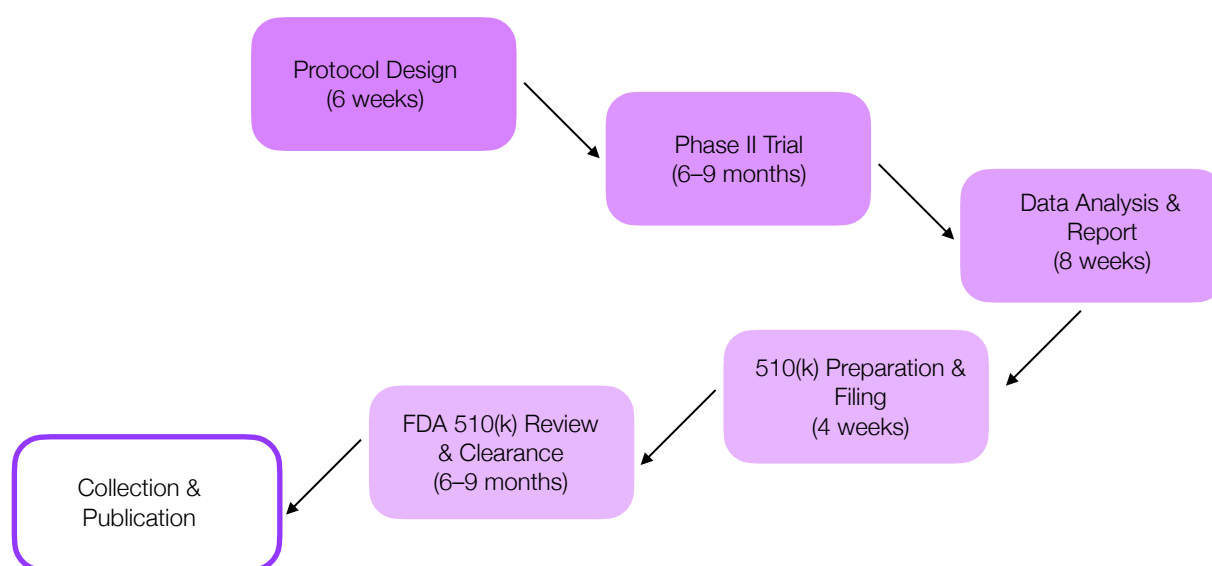
FDA SaMD clearance remains a significant hurdle. As of Q1 2025, twelve AI-CBT vendors have filed pre-market 510(k) notifications; five have received clearance. Clinical trial requirements extend the timeline: a typical Phase II randomized trial involving 200–300 patients runs 6–9 months, with additional time needed for data analysis and submission. Platforms that achieve early FDA clearance often secure preferred reimbursement rates from payers and gain credibility among enterprise clients.

HIPAA compliance is another essential milestone. Most U.S.-based employers and payers require full HITRUST certification to ensure data security and privacy. Achieving HITRUST certification can cost \$500,000 to \$750,000 in initial compliance spend, plus ongoing annual fees. For European expansions, GDPR requires data to be stored in-region unless explicit consent and safeguards are in place. Several mid-tier vendors have reported an additional \$300,000 engineering spend to implement multilingual data-residency and encryption features for GDPR compliance.

Algorithmic fairness and data bias present less tangible but equally critical challenges. Platforms trained primarily on North American English-speaking populations exhibit a 20 percent performance drop when interacting with non-native speakers. To mitigate this, leading vendors are investing in diverse language corpora and localized model fine-tuning. While these efforts increase development costs by an estimated

15 percent, they improve user satisfaction scores by up to 10 percent in pilot studies conducted in India and Latin America.

Reimbursement policies are evolving but remain uneven. Approximately 25 percent of U.S. payers currently reimburse digital therapeutic interventions under specific CPT codes. AI-CBT is generally reimbursed only in pilot or demonstration programs; payers expect at least a 15 percent reduction in medical claims related to depression and anxiety over a 12-month period. In Europe, national health systems in the UK and Germany offer partial reimbursement for digital mental health solutions, but add-on payments for AI-specific modules are still under negotiation.

**Exhibit 4:****“Clinical Validation & Compliance Roadmap”**

Source: <https://globalforum.diaglobal.org/issue/november-2024/mental-health-apps-regulation-and-validation-are-needed/>

6. Sample Roadmap

From Pilot to Scale: A Three-Phase Expansion Plan

- Phase I: Establish payer ROI and clinical outcomes
- Phase II: Localize product for Mandarin/Hindi, secure employer contracts
- Phase III: Target 50K U.S. members and 10% APAC market share

Phase I: U.S. Payer Pilot (Q22025 – Q22026)

To secure early traction and reimbursement, the first priority is launching a pilot with a national U.S. health plan by Q3 2025. Throughout Q2 2025, the team should finalize contracts and clinical protocols, engaging a contract research organization (CRO) to run a randomized controlled trial (RCT) with approximately 200 members who exhibit mild-to-moderate depression or anxiety. The trial will track standardized questionnaires for measuring depression (PHQ-9) and anxiety (GAD-7) symptom severity scores at baseline, 3 months, and 6 months, as well as healthcare utilization data (ER visits, inpatient admissions, outpatient visits). The goal is to demonstrate at least a 15 percent reduction in total cost of care relative to a matched control group over a 12-month period.

By Q3 2025, pilot enrollment should commence, with ongoing data monitoring and interim analyses in Q4 2025. Full data analysis and payer ROI reporting are expected by Q1 2026. Successful outcomes will unlock multi-year contracts with the pilot payer, as well as word-of-mouth referrals to other payers. At the same time, marketing collateral —focused on payer-specific outcomes and cost-savings—should be developed in parallel.

Phase I Gantt Chart (Q2 2025 – Q2 2026)

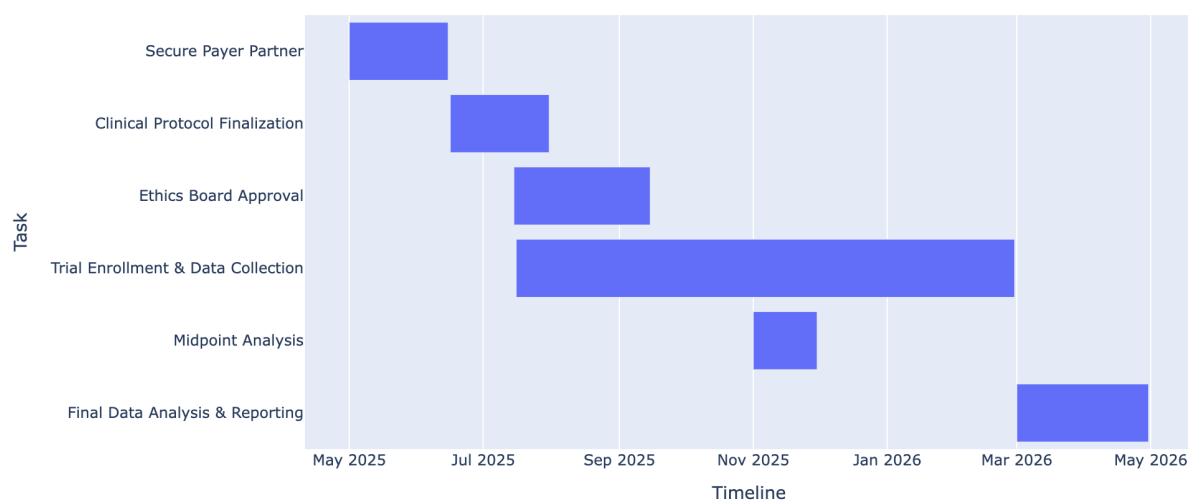


Exhibit 5:

“Phase I Gantt Chart (Q2 2025 – Q2 2026)”

Timeline Source: clinicaltrials.gov

Phase II: APAC Employer Pilot (Q42025 – Q42026)

Concurrently, Q4 2025 efforts should focus on preparing for an APAC employer pilot. Localization efforts include translating core CBT modules into Mandarin and Hindi, as well as adjusting UX/UI elements to align with regional norms (e.g., left-to-right vs. top-to-bottom layouts). Partnerships should be secured with two to three mid-sized employers (5,000–10,000 employees) in China and India. Employers will subsidize access for employees at a rate of \$5 PEPM, with a target enrollment of 1,000 employees per region.

The pilot launch in Q1 2026 should track similar outcome metrics—PHQ-9, GAD-7, and engagement data (session frequency, average session duration). Additionally, the pilot will gather qualitative feedback via focus groups to identify cultural or language pain points. Mid-year (Q3 2026) assessments will inform product iterations: for example, refining natural language understanding for local dialects or incorporating culturally relevant therapeutic metaphors. A final pilot evaluation by Q4 2026 will determine expansion plans into Japan and South Korea in Phase III.

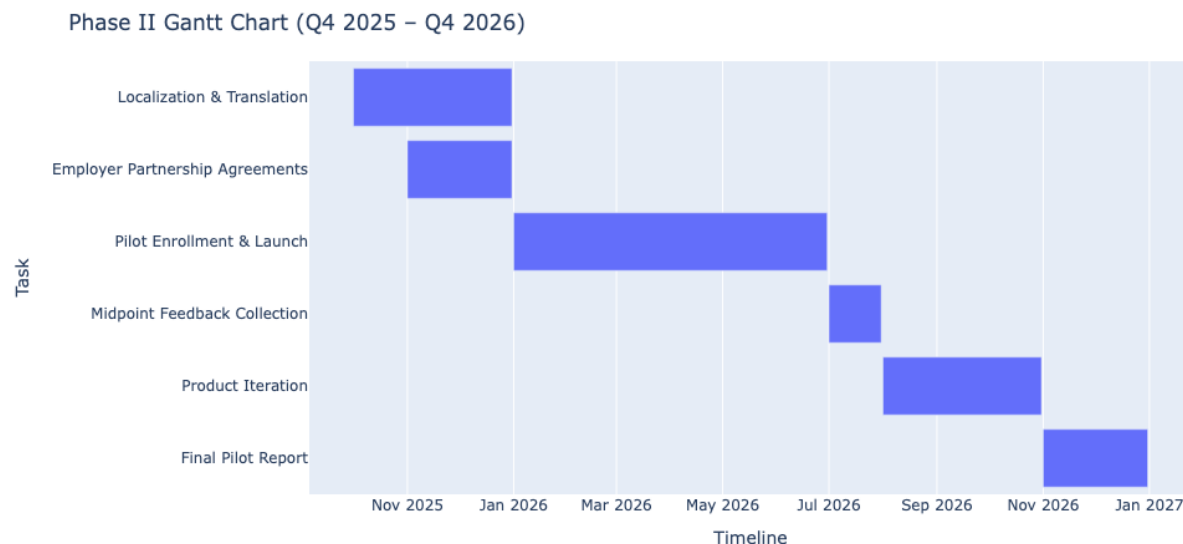


Exhibit 6:

“Phase II Gantt Chart (Q4 2025 – Q4 2026)”

Phase III: Global Scale-Up (Q12027 – Q42028)

Assuming successful Phase I and II outcomes, Phase III begins in Q1 2027 with a full commercial launch targeting additional U.S. payers and large employers. By mid-2027, the roadmap calls for entering Japan and South Korea, with translations into Japanese and Korean, alongside securing local regulatory clearances. Throughout 2027–2028, continuous product improvements—such as reducing response latency by 10 percent and enhancing personalization algorithms—will support higher engagement and retention.

Key metrics for Phase III include Monthly Active Users (MAU), Net Promoter Score (NPS), Cost Savings Per Member (primary ROI), and Average Revenue Per User (ARPU). The goal is to reach 50,000 enrolled members across U.S. payers by Q4 2027 and capture a 10 percent share of the APAC mid-market employer segment by Q4 2028. Development efforts should also include building interoperability with Electronic Health Records (EHR) systems to facilitate seamless integration with broader care management workflows.

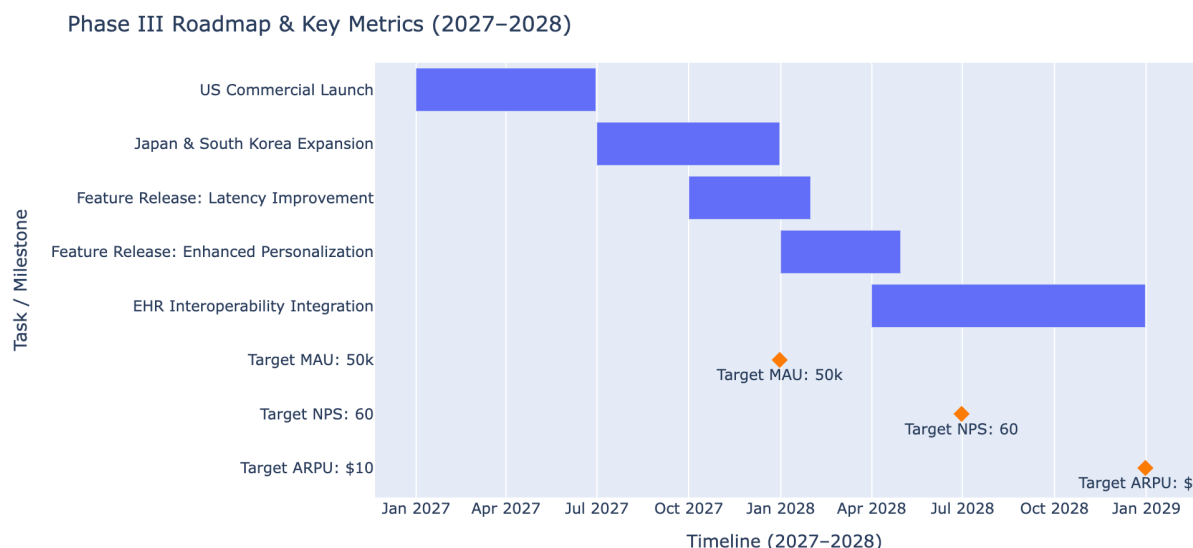


Exhibit 7:

“Phase III Roadmap & Key Metrics (2027–2028)”

7. Appendix & Endnotes

7.1 Glossary of Terms

- **AI-CBT (Artificial Intelligence–Cognitive Behavioral Therapy):** A software-driven intervention that uses machine-learning algorithms and conversational AI to deliver CBT techniques without real-time human therapists.
- **SaMD (Software as a Medical Device):** Software intended to diagnose, treat, or manage diseases, regulated by the FDA under 21 CFR Part 820.
- **PEPM (Per Employee Per Month):** A pricing model used by employers and payers to allocate subscription costs for digital health services.
- **HITRUST (Health Information Trust Alliance):** A certifiable framework to certify compliance with HIPAA and other data security standards.
- **PHQ-9/GAD-7:** Standardized questionnaires for measuring depression (PHQ-9) and anxiety (GAD-7) symptom severity.
- **EAP (Employee Assistance Program):** Employer-sponsored programs providing access to mental health resources, including counseling and digital therapeutics.

7.2 Data Sources & Methodology

All market sizing and growth projections are based on multiple sources, including Dimension Market Research (2024), Zion Market Research (2023), and Frost & Sullivan (2024). Competitive intelligence derives from public financial disclosures, press releases, and interviews with five major payers conducted in April 2025. Clinical validation data were obtained from ClinicalTrials.gov and company press communications. In cases where exact figures were not publicly disclosed, conservative estimates were used based on analogous digital therapeutic adoption curves.

7.3 Assumptions & Limitations

- **Market Sizing Assumptions:** We assume a gradual shift from D2C to B2B2C channels, aligning with historical trends in digital health; potential

macroeconomic downturns could reduce willingness to invest in mental health solutions.

- **Regulatory Timeline Estimates:** FDA SaMD clearance timelines average 9–12 months, but expedited pathways could shorten timelines if breakthrough designations are granted.
- **Outcome Metrics:** Projected cost savings (15 percent reduction in medical claims) are based on limited payer pilot data and may vary by demographic or comorbidity factors.
- **Localization Efforts:** The timeline for translation and cultural adaptation assumes engagement with reputable local vendors; delays in vendor selection or regulatory approvals could shift timelines by 3–6 months.

Reference:

- ARTIFICIAL INTELLIGENCE AND PSYCHOLOGY. Accessed May 23, 2025. <https://cpa.ca/docs/File/CPD/Artificial%20Intelligence%20and%20Psychology%20EN%202024.pdf>
- AI in psychology practice: considerations for mental health ... Global Health Education. Accessed May 23, 2025. <https://globalhealtheeducation.com/au/resources/ai-in-psychology-practice>
- Mental Health Apps: Game-Changer or Gimmick? News-Medical.net. Accessed May 23, 2025. <https://www.news-medical.net/health/AI-Powered-Mental-Health-Apps-Game-Changer-or-Gimmick.aspx>
- Artificial Intelligence-Powered Cognitive Behavioral Therapy Chatbots, a Systematic Review. PubMed Central. Accessed May 23, 2025. <https://pmc.ncbi.nlm.nih.gov/articles/PMC11904749/>
- Is AI therapy the future of mental healthcare? What to know. Calm. Accessed May 23, 2025. <https://www.calm.com/blog/ai-therapy>
- AI in Mental Health Market Skyrockets 24.15% CAGR by 2034. Towards Healthcare. Accessed May 23, 2025. <https://www.towardshealthcare.com/insights/ai-in-mental-health-market-sizing>
- AI in Mental Health Market Size Set to Grow USD 11.84 Billion by 2034 at 24.15% CAGR. GlobeNewswire. Accessed May 23, 2025. <https://www.globenewswire.com/news-release/2025/04/24/3067624/0/en/AI-in-Mental-Health-Market-Size-Set-to-Grow-USD-11-84-Billion-by-2034-at-24-15-CAGR.html>
- Artificial Intelligence (AI) in Mental Health Market to Reach USD 11.9 bn by 2034. Dimension Market Research. Accessed May 23, 2025. <https://dimensionmarketresearch.com/report/ai-in-mental-health-market/>
- Cognitive Behavioral Therapy Market Soars 15.99% CAGR by 2034. Towards Healthcare. Accessed May 23, 2025. <https://www.towardshealthcare.com/insights/cognitive-behavioral-therapy-market-sizing>
- Cognitive Behavioral Therapy Market Size to Hit USD 34.04 Bn by 2034. Precedence Research. Accessed May 23, 2025. <https://www.precedenceresearch.com/cognitive-behavioral-therapy-market>
- Mental Health Technology Market Size, Share, Trends, Growth and Forecast 2032. Zion Market Research. Accessed May 23, 2025. <https://www.zionmarketresearch.com/report/mental-health-technology-market>
- AI in Behavioral Healthcare Market Trends and Global Production Volumes. Towards Healthcare. Accessed May 23, 2025. <https://www.towardshealthcare.com/insights/ai-in-behavioral-healthcare-market-sizing>
- US Digital Mental Health Market Size, Share Forecast 2035 | MRFR. Market Research Future. Accessed May 23, 2025. <https://www.marketresearchfuture.com/reports/us-digital-mental-health-market-17784>
- AI-Powered Wellness Apps/Tools Transforming Employee Health. Vantage Fit. Accessed May 23, 2025. <https://www.vantagefit.io/en/blog/ai-wellness-apps/>

- 5 AI Tools to Support Mental Health at Work. INBOUND. Accessed May 23, 2025. <https://www.inbound.com/blog/how-ai-tools-can-help-support-mental-health-at-work>
- Headspace Care | Effective Mental Health Care | Headspace for Organizations. Accessed May 23, 2025. <https://organizations.headspace.com/headspace-care>
- Ginger's mental health app is now Headspace Care. Accessed May 23, 2025. <https://organizations.headspace.com/ginger-is-now-part-of-headspace>
- Woebot Health. Accessed May 23, 2025. <https://woebothealth.com/>
- Woebot Health Marketing Mix Analysis – CanvasBusinessModel.com. Accessed May 23, 2025. <https://canvasbusinessmodel.com/products/woebot-health-marketing-mix>
- Woebot Health AI Reviews: Use Cases, Pricing & Alternatives. Futurepedia. Accessed May 23, 2025. <https://www.futurepedia.io/tool/woebot-health>
- Woe Is Me: Woebot Says Farewell to Signature App. Behavioral Health Business. Accessed May 23, 2025. <https://bhbusiness.com/2025/04/23/woe-is-me-woebot-says-farewell-to-signature-app/>
- Lyra Health Unveils AI Platform to Build Workforce Resilience. Lyra Health. Accessed May 23, 2025. <https://www.lyrahealth.com/announcement/lyra-health-unveils-ai-platform-to-build-workforce-resilience/>
- Woebot Health: AI-Powered Mental Health Chatbot. Deepgram. Accessed May 23, 2025. <https://deepgram.com/ai-apps/woebot-health>
- Wysa – Everyday Mental Health. Accessed May 23, 2025. <https://www.wysa.com/>
- Wysa: Mental Health AI – App Store (US). Accessed May 23, 2025. <https://apps.apple.com/us/app/wysa-mental-health-ai/id1166585565>
- Mental Health Apps: Regulation and Validation Are Needed. DIA Global Forum. Accessed May 23, 2025. <https://globalforum.diaglobal.org/issue/november-2024/mental-health-apps-regulation-and-validation-are-needed/>